

COMMITTEE ON HEALTH & HUMAN SERVICES
HOUSE OF REPRESENTATIVES AMENDMENTS TO H.B. 2434
(Reference to printed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

[~~GREEN STRIKEOUT IN BRACKETS~~] indicates new text removed from statute or previously enacted session law.

[~~Green strikeout in brackets~~] indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<~~Green strikeout in carets~~>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Section 36-2525, Arizona Revised Statutes, is amended to
3 read:

4 36-2525. Prescription orders; labels; recordkeeping;
5 definition

6 A. In addition to the requirements of section 32-1968 pertaining to
7 prescription orders for prescription-only drugs, the prescription order
8 for a controlled substance shall bear the name, address and federal
9 registration number of the prescriber. A prescription order for a
10 schedule II controlled substance drug other than a hospital drug order for
11 a hospital inpatient shall contain only one drug order per prescription
12 blank. If authorized verbally by the prescriber, the pharmacist may make
13 changes to a written or electronic schedule II controlled substance
14 prescription order, except for any of the following:

- 15 1. The patient's name.
- 16 2. The prescriber's name.
- 17 3. The drug name.

18 B. The pharmacist must document on the original prescription order
19 the changes that were made pursuant to the verbal authorization and record
20 the time and date the authorization was granted.

21 C. A person who is registered to dispense controlled substances
22 under this chapter must keep and maintain prescription orders for
23 controlled substances as follows:

- 24 1. Prescription orders for controlled substances listed in
25 schedules I and II must be maintained in a separate prescription file for
26 controlled substances listed in schedules I and II only.
- 27 2. Prescription orders for controlled substances listed in
28 schedules III, IV and V must be maintained either in a separate
29 prescription file for controlled substances listed in schedules III, IV
30 and V only or in a form that allows them to be readily retrievable from

1 the other prescription records of the registrant. For the purposes of
2 this paragraph, "readily retrievable" means that, when the prescription is
3 initially filed, the face of the prescription is stamped in red ink in the
4 lower right corner with the letter "C" in a font that is at least one inch
5 high and that the prescription is filed in the usual consecutively
6 numbered prescription file for noncontrolled substance prescriptions. The
7 requirement to stamp the hard copy prescription with a red "C" is waived
8 if a registrant employs an electronic data processing system or other
9 electronic recordkeeping system for prescriptions that allows
10 identification by prescription number and retrieval of original documents
11 by the prescriber's name, patient's name, drug dispensed and date filled.

12 D. Except in emergency situations in conformity with subsection E
13 of this section, under the conditions specified in subsections F and G of
14 this section or when dispensed directly by a medical practitioner to an
15 ultimate user, a controlled substance in schedule II shall not be
16 dispensed without either the written prescription order in ink or
17 indelible pencil or typewritten and manually signed by the medical
18 practitioner or an electronic prescription order as prescribed by federal
19 law or regulation. A schedule II controlled substance that is an opioid
20 may be dispensed only with an electronic prescription order as prescribed
21 by federal law or regulation. A prescription order for a schedule II
22 controlled substance shall not be dispensed more than ninety days after
23 the date on which the prescription order was issued. Notwithstanding any
24 other provision of this section, a pharmacy may sell and dispense a
25 schedule II controlled substance prescribed by a medical practitioner who
26 is located in another state if the prescription was issued to the patient
27 according to and in compliance with the applicable laws of the state of
28 the prescribing medical practitioner and federal law. A prescription
29 order for a schedule II controlled substance shall not be refilled. A
30 pharmacist is not in violation of this subsection and may dispense a
31 prescription order in the following circumstances:

32 1. During any time period in which an established electronic
33 prescribing system or a pharmacy management system is not operational or
34 available in a timely manner. If the electronic prescribing system or a
35 pharmacy management system is not operational or available, the pharmacist
36 may dispense a prescription order that is written for a schedule II
37 controlled substance that is an opioid. The pharmacist must maintain a
38 record, for a period of time prescribed by the board, of when the
39 electronic prescribing system or pharmacy management system is not
40 operational or available in a timely manner.

41 2. The prescription order for a schedule II controlled substance
42 that is an opioid is in writing and indicates that the medical
43 practitioner who issued the prescription order provided care for the
44 patient in a veterans administration facility, a health facility on a
45 military base, an Indian health services hospital or other Indian health
46 service facility, or a tribal-owned clinic.

1 E. In emergency situations, emergency quantities of schedule II
2 controlled substances may be dispensed on an oral prescription order of a
3 medical practitioner. Such an emergency prescription order shall be
4 immediately reduced to writing by the pharmacist and shall contain all the
5 information required for schedule II controlled substances except for the
6 manual signing of the order by the medical practitioner. Within seven
7 days after authorizing an emergency oral prescription order, the
8 prescribing medical practitioner shall cause a written prescription order
9 manually signed for the emergency quantity prescribed to be delivered to
10 the dispensing pharmacist or an electronic prescription order to be
11 transmitted to the dispensing pharmacist. In addition to conforming to
12 other requirements for prescription orders for schedule II controlled
13 substances, the prescription order shall indicate electronically or have
14 written on its face "authorization for emergency dispensing" and the date
15 of the oral order. If the prescribing medical practitioner fails to
16 deliver such an emergency prescription order within seven days in
17 conformance with board rules, the pharmacist shall notify the board.
18 Failure of the pharmacist to notify the board voids the authority
19 conferred by this subsection to dispense without a prescription order of a
20 medical practitioner that is electronic or that is written and manually
21 signed.

22 F. Notwithstanding subsections D and N of this section, a patient's
23 medical practitioner or the medical practitioner's agent may transmit to a
24 pharmacy by fax a prescription order written for a schedule II controlled
25 substance, including opioids, if the prescription order is any of the
26 following:

27 1. To be compounded for the direct administration to a patient by
28 parenteral, intravenous, intramuscular, subcutaneous or intraspinal
29 infusion.

30 2. For a resident of a long-term care facility.

31 3. For a patient who is enrolled in a hospice care program that is
32 certified or paid for by medicare under title XVIII or a hospice program
33 that is licensed by this state. The medical practitioner or the medical
34 practitioner's agent must note on the prescription that the patient is a
35 hospice patient.

36 G. A fax transmitted pursuant to subsection F of this section is
37 the original written prescription order for purposes of this section and
38 must be maintained as required by subsection C of this section.

39 H. Except when dispensed directly by a medical practitioner to an
40 ultimate user, a controlled substance included in schedule III or IV that
41 requires a prescription order as determined under state or federal laws
42 shall not be dispensed without a written or oral prescription order of a
43 medical practitioner or an electronic prescription order as prescribed by
44 federal law or regulation. The prescription order shall not be filled or
45 refilled more than six months after the date on which the prescription
46 order was issued. A prescription order authorized to be refilled shall
47 not be refilled more than five times. Additional quantities may only be

1 authorized by the prescribing medical practitioner through issuance of a
2 new prescription order that shall be treated by the pharmacist as a new
3 and separate prescription order.

4 I. Except when dispensed directly by a medical practitioner to an
5 ultimate user, a controlled substance that is included in schedule V and
6 that requires a prescription order as determined under state or federal
7 laws shall not be dispensed without a written or oral prescription order
8 of a medical practitioner. The prescription order may be refilled as
9 authorized by the prescribing medical practitioner but shall not be filled
10 or refilled more than one year after the date of issuance.

11 J. A controlled substance that is listed in schedule III, IV or V
12 and that does not require a prescription order as determined under state
13 or federal laws may be dispensed at retail by a pharmacist or a pharmacy
14 intern under the pharmacist's supervision without a prescription order to
15 a purchaser who is at least eighteen years of age if all of the following
16 are true:

17 1. It is for a legitimate medical purpose.

18 2. Not more than two hundred forty cubic centimeters (eight ounces)
19 of any such controlled substance containing opium, nor more than one
20 hundred twenty cubic centimeters (four ounces) of any other such
21 controlled substance, nor more than forty-eight dosage units of any such
22 controlled substance containing opium, nor more than twenty-four dosage
23 units of any other controlled substance may be dispensed at retail to the
24 same purchaser in any given forty-eight-hour period.

25 3. Not more than one hundred dosage units of any single active
26 ingredient ephedrine preparation may be sold, offered for sale, bartered
27 or given away to any one person in any one thirty-day period.

28 4. The pharmacist or pharmacy intern requires every purchaser of a
29 controlled substance under this subsection who is not known to that person
30 to furnish suitable identification, including proof of age if appropriate.

31 5. A bound record book for dispensing controlled substances under
32 this subsection is maintained by the pharmacist and contains the name and
33 address of the purchaser, the name and quantity of the controlled
34 substance purchased, the date of each purchase and the name or initials of
35 the pharmacist or pharmacy intern who dispensed the substance to the
36 purchaser. The book shall be maintained in conformity with the
37 recordkeeping requirements of section 36-2523.

38 K. In the absence of a law requiring a prescription for a
39 schedule V controlled substance, the board, by rules, may require, or
40 remove the requirement of, a prescription order for a schedule V
41 controlled substance.

42 L. The label on a container of a controlled substance that is
43 directly dispensed by a medical practitioner or pharmacist and that is not
44 for the immediate administration to the ultimate user, such as a bed
45 patient in a hospital, shall bear the name and address of the dispensing
46 medical practitioner or pharmacist, the serial number, the date of
47 dispensing, the name of the prescriber, the name of the patient or, if an

1 animal, the name of the owner of the animal and the species of the animal,
2 the directions for use and cautionary statements, if any, contained in the
3 prescription order or required by law. If the controlled substance is
4 included in schedule II, III or IV, the label shall bear a transfer
5 warning to the effect: "Caution: federal law prohibits the transfer of
6 this drug to any person other than the patient for whom it was
7 prescribed". The container of a schedule II controlled substance that is
8 an opioid that is directly dispensed by a pharmacist and that is not for
9 the immediate administration to the ultimate user shall have a warning
10 label prescribed by the board about potential addiction.

11 M. Controlled substances in schedules II, III, IV and V may be
12 dispensed as electronically transmitted prescriptions if the prescribing
13 medical practitioner is all of the following:

14 1. Properly registered by the United States drug enforcement
15 administration.

16 2. Licensed in good standing in the United States jurisdiction in
17 which the medical practitioner practices.

18 3. Authorized to issue such prescriptions in the jurisdiction in
19 which the medical practitioner is licensed.

20 N. Notwithstanding any other provision of this section, each
21 prescription order, except a prescription order under subsection F of this
22 section, that is issued by a medical practitioner for a schedule II
23 controlled substance that is an opioid shall be transmitted electronically
24 to the dispensing pharmacy. A medical practitioner is not in violation of
25 this subsection:

26 1. During any time in which an established electronic prescribing
27 system or a pharmacy management system is not operational or available in
28 a timely manner. If the electronic prescribing system or a pharmacy
29 management system is not operational or available, the medical
30 practitioner may write a prescription order for a schedule II controlled
31 substance that is an opioid. The medical practitioner shall indicate on
32 the written prescription order that the electronic prescribing system or
33 pharmacy management system is not operational or available. The medical
34 practitioner must maintain a record, for a period of time prescribed by
35 the board, of when the electronic prescribing system or pharmacy
36 management system is not operational or available in a timely manner.

37 2. If the medical practitioner writes a prescription order for a
38 schedule II controlled substance that is an opioid that will be dispensed
39 for the patient from a veterans administration facility, a health facility
40 on a military base, an Indian health services hospital or other Indian
41 health service facility, or a tribal-owned clinic.

42 O. The requirement in subsections D and N of this section for an
43 electronic prescription order does not apply to a prescription order for a
44 schedule II controlled substance that is an opioid that is issued for
45 medication-assisted treatment for a substance use disorder.

46 P. The board, by rule, may provide additional requirements for
47 prescribing and dispensing controlled substances.

1 Q. In consultation with the ~~task force~~ COMPLIANCE WORKGROUP
2 established pursuant to section 36-2603, the board may prescribe by rule
3 additional exceptions to the electronic prescribing requirements specified
4 in this section for both pharmacists and medical practitioners.

5 R. Notwithstanding subsections D and N of this section, a medical
6 practitioner who is licensed pursuant to title 32, chapter 21 is not
7 required to comply with the electronic prescribing requirements of
8 subsections D and N of this section until the Arizona state veterinary
9 medical examining board determines that electronic prescribing software is
10 widely available for veterinarians and notifies the Arizona state board of
11 pharmacy of that determination.

12 S. For the purposes of this section, "medication-assisted
13 treatment" has the same meaning prescribed in section 32-3201.01.

14 Sec. 2. Section 36-2602, Arizona Revised Statutes, is amended to
15 read:

16 36-2602. Controlled substances prescription monitoring
17 program; contracts; retention and maintenance of
18 records

19 A. The board shall adopt rules to establish a controlled substances
20 prescription monitoring program. The program shall:

21 1. Be operated, monitored and maintained by the board.

22 2. Be staffed by the board.

23 3. Include a computerized central database tracking system to track
24 the prescribing, ~~AND~~ dispensing ~~and consumption~~ of schedule II, III, IV
25 and V controlled substances that are dispensed by a medical practitioner
26 or by a pharmacy that holds a valid license or permit issued pursuant to
27 title 32. The database shall include data from the department of health
28 services that identifies residents of this state who possess a registry
29 identification card issued pursuant to chapter 28.1 of this title. The
30 tracking system shall not interfere with the legal use of a controlled
31 substance for managing severe or intractable pain.

32 4. Assist law enforcement to identify illegal activity related to
33 prescribing, ~~AND~~ dispensing ~~and consuming~~ schedule II, III, IV and V
34 controlled substances.

35 5. Provide information to patients, medical practitioners and
36 pharmacists to help avoid the inappropriate use of schedule II, III, IV
37 and V controlled substances.

38 6. Be designed to minimize inconvenience to patients, prescribing
39 medical practitioners and pharmacies while effectuating the collection and
40 storage of information.

41 B. The board may enter into private or public contracts, including
42 intergovernmental agreements pursuant to title 11, chapter 7, article 3,
43 to ensure the effective operation of the program. Each contractor must
44 comply with the confidentiality requirements prescribed in this article
45 and is subject to the criminal penalties prescribed in section 36-2610.

46 C. The board shall maintain the following records for the following
47 periods of time:

1 1. A record of dispensing a controlled substance for seven years
2 after the date the controlled substance was dispensed.

3 2. ~~Affidavits~~ SEARCH WARRANTS for the purpose of an open
4 investigation by law enforcement for two years.

5 3. Court orders requesting medical record information in the
6 program for two years.

7 4. A patient's request of the patient's own prescription history
8 for two years.

9 5. A prescriber report for two years.

10 Sec. 3. Section 36-2603, Arizona Revised Statutes, is amended to
11 read:

12 36-2603. Compliance workgroup: membership: duties

13 A. The board shall appoint a ~~task force to help it administer the~~
14 ~~computerized central database tracking system, to identify educational,~~
15 ~~outreach and support services to advance medical practitioners' adoption~~
16 ~~of electronic prescribing of schedule II controlled substances and~~
17 ~~pharmacy implementation of section 36-2525 and to consult with regarding~~
18 ~~recommendations for exceptions to the electronic prescribing requirements~~
19 ~~prescribed in section 36-2525~~ COMPLIANCE WORKGROUP TO IDENTIFY POTENTIAL
20 NONCOMPLIANCE WITH THE REQUIREMENT TO USE THE CONTROLLED SUBSTANCES
21 PRESCRIPTION MONITORING PROGRAM. The chairperson of the board shall ~~chair~~
22 ~~SERVE AS CHAIRPERSON OF~~ the ~~task force~~ WORKGROUP. The ~~task force~~
23 WORKGROUP shall include the following members:

24 1. Pharmacists, medical practitioners and other licensed health
25 care providers.

26 2. Representatives of professional societies and associations for
27 pharmacists, medical practitioners and other licensed health care
28 providers.

29 3. Representatives of professional licensing boards.

30 4. Representatives of the Arizona health care cost containment
31 system administration.

32 ~~5. Representatives of state and federal agencies that have an~~
33 ~~interest in controlling controlled substances.~~

34 ~~6. Criminal prosecutors.~~

35 ~~7. Representatives of a health information organization in this~~
36 ~~state.~~

37 B. The ~~task force~~ WORKGROUP shall meet to establish the procedures
38 and conditions relating to the release of prescription information
39 pursuant to section 36-2604. The ~~task force~~ WORKGROUP shall meet at least
40 once each year and at the call of the chairperson.

41 C. ~~Task force~~ WORKGROUP members serve at the pleasure of the board
42 and are not eligible to receive compensation or reimbursement of expenses.

1 Sec. 4. Section 36-2604, Arizona Revised Statutes, is amended to
2 read:

3 36-2604. Use and release of confidential information:
4 definitions

5 A. Except as otherwise provided in this section, prescription
6 information submitted to the board pursuant to this article is
7 confidential and is not subject to public inspection. The board shall
8 establish procedures to ensure THAT the privacy and confidentiality of
9 ~~patients and that patient information~~ ALL DISPENSING DATA that is
10 collected, recorded and transmitted pursuant to this article is not
11 disclosed except as prescribed in this section.

12 B. The board or its designee shall review the prescription
13 information collected pursuant to this article. If the board or its
14 designee has reason to believe an act of unprofessional or illegal conduct
15 has occurred, the board or its designee shall notify the appropriate
16 professional licensing board. The board may delegate the duties
17 prescribed in this subsection to the executive director pursuant to
18 section 32-1904.

19 C. The board may release data collected by the program to the
20 following:

21 1. A person who is authorized to prescribe or dispense controlled
22 substances, or a delegate who is authorized by the prescriber or
23 dispenser, to assist that person to provide medical or pharmaceutical care
24 to a patient or to evaluate a patient or to assist with or verify
25 compliance with the requirements of this chapter, the rules adopted
26 pursuant to this chapter and the rules adopted by the department of health
27 services to reduce opioid overdose and death.

28 2. An individual who requests the individual's own prescription
29 monitoring information pursuant to section 12-2293.

30 3. A medical practitioner regulatory board established pursuant to
31 title 32, chapter 7, 11, 13, 14, 15, 16, 17, 18, 25 or 29.

32 4. A local, state or federal law enforcement or criminal justice
33 agency. The board shall provide this information only if the requesting
34 agency has a valid search warrant and is using the information for an open
35 investigation or complaint.

36 5. The Arizona health care cost containment system administration
37 and contractors regarding persons who are receiving services pursuant to
38 chapters 29 and 34 of this title or title XVIII of the social security
39 act. Except as required pursuant to subsection B of this section, the
40 board shall provide this information only if the administration or a
41 contractor ~~states in writing~~ ATTESTS that the information is necessary for
42 an open investigation or complaint or for performing a drug utilization
43 review for controlled substances that supports the prevention of opioid
44 overuse or abuse and the safety and quality of care provided to the
45 member.

46 6. A health care insurer. Except as required pursuant to
47 subsection B of this section, the board shall provide this information

1 only if the health care insurer ~~states in writing~~ ATTESTS that the
2 information is necessary for an open investigation or complaint or for
3 performing a drug utilization review for controlled substances that
4 supports the prevention of opioid overuse or abuse and the safety and
5 quality of care provided to the insured.

6 7. A person who is serving a lawful order of a court of competent
7 jurisdiction.

8 8. A person who is authorized to prescribe or dispense controlled
9 substances and who performs an evaluation on an individual pursuant to
10 section 23-1026.

11 9. A county medical examiner or alternate medical examiner who is
12 directing an investigation into the circumstances surrounding a death as
13 described in section 11-593 or a delegate who is authorized by the county
14 medical examiner or alternate medical examiner.

15 10. The department of health services regarding persons who are
16 receiving or prescribing controlled substances in order to implement a
17 public health response to address opioid overuse or abuse, including a
18 review pursuant to section 36-198. Except as required pursuant to
19 subsection B of this section, the board shall provide this information
20 only if the department states in writing that the information is necessary
21 to implement a public health response to help combat opioid overuse or
22 abuse.

23 D. Data provided by the board pursuant to this section may not be
24 used for any of the following:

- 25 1. Credentialing health care professionals.
- 26 2. Determining payment.
- 27 3. Preemployment screening.
- 28 4. Any purpose other than as specified in this section.

29 E. For a fee determined by the board, the board may provide data to
30 public or private entities for statistical, research or educational
31 purposes after removing ANY PERSONAL IDENTIFYING information ~~that could be~~
32 ~~used to identify individual patients or persons who received prescriptions~~
33 ~~from dispensers.~~

34 F. Any employee of the administration, a contractor or a health
35 care insurer who is assigned delegate access to the program shall operate
36 under the authority and responsibility of the administration's,
37 contractor's or health care insurer's chief medical officer or other
38 employee who is a licensed health care professional and who is authorized
39 to prescribe or dispense controlled substances. A delegate of the
40 administration, a contractor or a health care insurer shall hold a valid
41 license or certification issued pursuant to title 32, chapter 7, 11, 13,
42 14, 15, 16, 17, 18, 19.1, 25, 29 or 33 as a condition of being assigned
43 and provided delegate access to the program by the board. Each employee
44 of the administration, a contractor or a health care insurer who is a
45 licensed health care professional and who is authorized to prescribe or
46 dispense controlled substances may authorize not more than ten delegates.

1 G. If, after reviewing the information provided pursuant to
2 subsection C, paragraph 4 of this section, an investigator finds no
3 evidence of a statutory crime but suspects a medical practitioner of
4 prescribing controlled substances inappropriately in manner or amount, the
5 investigator may refer the medical practitioner to the relevant
6 professional licensing board for investigation of possible deviation from
7 the standard of care but may not arrest or otherwise undertake criminal
8 proceedings against the medical practitioner.

9 H. A person who is authorized to prescribe or dispense controlled
10 substances or the chief medical officer or other licensed health care
11 professional of the administration, a contractor or a health care insurer
12 who is authorized to prescribe or dispense controlled substances shall
13 deactivate a delegate within five business days after an employment status
14 change, the request of the delegate or the inappropriate use of the
15 controlled substances prescription monitoring program's central database
16 tracking system.

17 I. For the purposes of this section:

18 1. "Administration" and "contractor" have the same meanings
19 prescribed in section 36-2901.

20 2. "Delegate" means any of the following:

21 (a) A licensed health care professional who is employed ~~in~~ BY the
22 office of or ~~in~~ BY a hospital with the prescriber or dispenser.

23 (b) An unlicensed medical records technician, medical assistant or
24 office manager who is employed ~~in~~ BY the office of or ~~in~~ BY a hospital
25 with the prescriber or dispenser and who has received training regarding
26 both the health insurance portability and accountability act privacy
27 standards (45 Code of Federal Regulations part 164, subpart E) and
28 security standards (45 Code of Federal Regulations part 164, subpart C).

29 (c) A forensic pathologist, medical death investigator or other
30 qualified person who is assigned duties in connection with a death
31 investigation pursuant to section 11-594.

32 (d) A registered pharmacy technician trainee, licensed pharmacy
33 technician or licensed pharmacy intern who ~~works in a facility with~~ IS
34 EMPLOYED BY THE PHARMACY OF OR BY A HOSPITAL OF the dispenser IN THIS
35 STATE.

36 (e) Any employee of the administration, a contractor or a health
37 care insurer who is authorized by the administration's, contractor's or
38 health care insurer's chief medical officer or other licensed health care
39 professional who is authorized to prescribe or dispense controlled
40 substances.

41 3. "Health care insurer" has the same meaning prescribed in section
42 20-3151.

1 Sec. 5. Section 36-2606, Arizona Revised Statutes, is amended to
2 read:

3 36-2606. Registration; access; requirements; mandatory use;
4 annual user satisfaction survey; report;
5 definitions

6 A. A medical practitioner regulatory board shall notify each
7 medical practitioner who receives an initial or renewal license and who
8 intends to apply for registration or has an active registration under the
9 controlled substances act (21 United States Code sections 801 through 904)
10 WITH AN ADDRESS IN THIS STATE of the medical practitioner's responsibility
11 to register with the Arizona state board of pharmacy and be granted access
12 to the controlled substances prescription monitoring program's central
13 database tracking system. The Arizona state board of pharmacy shall
14 provide access to the central database tracking system to each medical
15 practitioner who has a valid license pursuant to title 32 and who
16 possesses ~~an Arizona~~ A registration under the controlled substances act
17 (21 United States Code sections 801 through 904) WITH AN ADDRESS IN THIS
18 STATE. The Arizona state board of pharmacy shall notify each pharmacist
19 of the pharmacist's responsibility to register with the Arizona state
20 board of pharmacy and be granted access to the controlled substances
21 prescription monitoring program's central database tracking system. The
22 Arizona state board of pharmacy shall provide access to the central
23 database tracking system to each pharmacist who has a valid license
24 pursuant to title 32, chapter 18 and who is employed by either:

25 1. A facility that has a valid United States drug enforcement
26 administration registration number.

27 2. The administration, a contractor or a health care insurer and
28 who has a national provider identifier number.

29 B. The registration is:

30 1. Valid in conjunction with a valid United States drug enforcement
31 administration registration number and a valid license issued by a medical
32 practitioner regulatory board established pursuant to title 32, chapter 7,
33 11, 13, 14, 15, 16, 17, 25 or 29.

34 2. Valid in conjunction with a valid license issued by the Arizona
35 state board of pharmacy for a pharmacist who is employed by either:

36 (a) A facility that has a valid United States drug enforcement
37 administration registration number.

38 (b) The administration, a contractor or a health care insurer and
39 who has a national provider identifier number.

40 3. Not transferable or assignable.

41 C. An applicant for registration pursuant to this section must
42 apply as prescribed by the board.

43 ~~D. Pursuant to a fee prescribed by the board by rule, the board may~~
44 ~~issue a replacement registration to a registrant who requests a~~
45 ~~replacement because the original was damaged or destroyed, because of a~~
46 ~~change of name or for any other good cause as prescribed by the board.~~

1 ~~F.~~ D. A person who is authorized to access the controlled
2 substances prescription monitoring program's central database tracking
3 system may do so using only that person's assigned identifier and may not
4 use the assigned identifier of another person.

5 ~~F.~~ E. ~~Beginning the later of October 1, 2017 or sixty days after~~
6 ~~the statewide health information exchange has integrated the controlled~~
7 ~~substances prescription monitoring program data into the exchange,~~ A
8 medical practitioner, before prescribing an opioid analgesic or
9 benzodiazepine controlled substance listed in schedule II, III or IV for a
10 patient, shall obtain a patient utilization report regarding the patient
11 for the preceding twelve months from the controlled substances
12 prescription monitoring program's central database tracking system ~~at the~~
13 ~~beginning of each new course of treatment and at least quarterly while~~
14 ~~that prescription remains a part of the treatment~~ BEFORE ISSUING A
15 PRESCRIPTION AND BEFORE PRESCRIBING ANY SUBSEQUENT REFILL OF THAT
16 PRESCRIPTION. Each medical practitioner regulatory board shall notify the
17 medical practitioners licensed by that board of the ~~applicable date.~~ A
18 ~~medical practitioner may be granted a one-year waiver from the requirement~~
19 ~~in this subsection due to technological limitations that are not~~
20 ~~reasonably within the control of the practitioner or other exceptional~~
21 ~~circumstances demonstrated by the practitioner, pursuant to a process~~
22 ~~established by rule by the Arizona state board of pharmacy~~ MANDATORY USE
23 REQUIREMENTS OUTLINED IN THIS SUBSECTION.

24 ~~G.~~ F. Before a pharmacist dispenses or before a pharmacy
25 technician or pharmacy intern of a remote dispensing site pharmacy
26 dispenses a schedule II controlled substance, a dispenser shall obtain a
27 patient utilization report regarding the patient for the preceding twelve
28 months from the controlled substances prescription monitoring program's
29 central database tracking system ~~at the beginning of each new course of~~
30 ~~treatment.~~

31 ~~H.~~ G. The medical practitioner or dispenser is not required to
32 obtain a patient utilization report from the central database tracking
33 system pursuant to subsection ~~F.~~ E of this section if any of the following
34 applies:

35 1. The patient is receiving hospice care or palliative care for a
36 serious or chronic illness.

37 2. The patient is receiving care for cancer, a cancer-related
38 illness or condition or dialysis treatment.

39 3. A medical practitioner will administer the controlled substance.

40 4. The patient is receiving the controlled substance during the
41 course of inpatient or residential treatment in a hospital, nursing care
42 facility, assisted living facility, correctional facility or mental health
43 facility.

44 5. The medical practitioner is prescribing the controlled substance
45 to the patient for not more than a five-day period for an invasive medical
46 or dental procedure or a medical or dental procedure that results in acute
47 pain to the patient.

1 6. The medical practitioner is prescribing the controlled substance
2 to the patient for not more than a five-day period for a patient who has
3 suffered an acute injury or a medical or dental disease process that is
4 diagnosed in an emergency department setting and that results in acute
5 pain to the patient. An acute injury or medical disease process does not
6 include back pain.

7 ~~F.~~ H. On or before December 31, 2026, a vendor that provides
8 electronic medical records services to a medical practitioner in this
9 state shall integrate the vendor's electronic medical records system with
10 the program's central database tracking system either directly or through
11 the statewide health information exchange or a third-party vendor.

12 ~~G.~~ I. If a medical practitioner or dispenser uses electronic
13 medical records that integrate data from the controlled substances
14 prescription monitoring program, a review of the electronic medical
15 records with the integrated data shall be deemed compliant with the review
16 of the program's central database tracking system as required in
17 subsection ~~F~~ E of this section.

18 ~~K.~~ J. The board shall promote and enter into data sharing
19 agreements to integrate and display patient utilization reports within
20 electronic medical records.

21 ~~L.~~ K. By complying with this section, a medical practitioner or
22 dispenser who acts in good faith, or the medical practitioner's or
23 dispenser's employer, is not subject to liability or disciplinary action
24 arising solely from either:

25 1. Requesting or receiving, or failing to request or receive,
26 prescription monitoring data from the program's central database tracking
27 system.

28 2. Acting or failing to act on the basis of the prescription
29 monitoring data provided by the program's central database tracking
30 system.

31 ~~M.~~ L. Notwithstanding any provision of this section to the
32 contrary, medical practitioners or dispensers and their delegates are not
33 in violation of this section during any time period in which the
34 controlled substances prescription monitoring program's central database
35 tracking system is suspended or is not operational or available in a
36 timely manner. If the program's central database tracking system is not
37 accessible, the medical practitioner or dispenser or the medical
38 practitioner's or dispenser's delegate shall document the date and time
39 the practitioner, dispenser or delegate attempted to use the central
40 database tracking system pursuant to a process established by board rule.

41 ~~N.~~ M. The board shall conduct an annual voluntary survey of
42 program users to assess user satisfaction with the program's central
43 database tracking system. The survey may be conducted electronically. On
44 or before December 1 of each year, the board shall provide a report of the
45 survey results to the president of the senate, the speaker of the house of
46 representatives and the governor and shall provide a copy of this report
47 to the secretary of state.

1 ~~0.~~ N. This section does not prohibit a medical practitioner
2 regulatory board or the Arizona state board of pharmacy from obtaining and
3 using information from the program's central database tracking system.

4 ~~P.~~ 0. For the purposes of this section:

5 1. "Administration" has the same meaning prescribed in section
6 36-2901.

7 2. "Contractor" has the same meaning prescribed in section 36-2901.

8 3. "Dispenser" means a pharmacist who is licensed pursuant to title
9 32, chapter 18.

10 4. "Emergency department" means the unit within a hospital that is
11 designed to provide emergency services.

12 5. "Health care insurer" has the same meaning prescribed in section
13 20-3151.

14 Sec. 6. Section 36-2608, Arizona Revised Statutes, is amended to
15 read:

16 36-2608. Reporting requirements; exceptions

17 A. If a medical practitioner OR PHARMACY dispenses a controlled
18 substance listed in section 36-2513, 36-2514, 36-2515 or 36-2516 or the
19 rules adopted pursuant to chapter 27, article 2 of this title, or if a
20 prescription for a controlled substance listed in any of those sections
21 that is approved by the United States food and drug administration is
22 dispensed by a pharmacy in this state, a health care facility in this
23 state for outpatient use or a board-permitted nonresident pharmacy for
24 delivery to a person residing in this state, the medical practitioner,
25 health care facility or pharmacy must report the following information as
26 applicable and as prescribed by the board by rule:

27 1. The name, address, telephone number, prescription number and
28 United States drug enforcement administration controlled substance
29 registration number of the dispenser.

30 2. The name, address and date of birth of the person for whom the
31 prescription is written.

32 3. The name, address, telephone number and United States drug
33 enforcement administration controlled substance registration number of the
34 prescribing medical practitioner.

35 4. The name, strength, quantity, dosage and national drug code
36 number of the schedule II, III, IV or V controlled substance dispensed.

37 5. [THROUGH JUNE 30, 2027,] the date the prescription was
38 [dispensed] [FILLED]. [BEGINNING JULY 1, 2027, THE DATE THE PRESCRIPTION
39 WAS FILLED.]

40 6. [BEGINNING JULY 1, 2027,] THE DATE THE PRESCRIPTION WAS SOLD TO
41 THE ULTIMATE USER OR THE ULTIMATE USER'S AGENT.

42 ~~6.~~ 7. The number of refills, if any, authorized by the medical
43 practitioner.

44 B. ~~Except as provided in subsection D of this section, A dispenser~~
45 ~~must use the latest version of the standard implementation guide for~~
46 ~~prescription monitoring programs published by the American society for~~

1 ~~automation in pharmacy~~ THIS STATE'S DATA SUBMISSION GUIDE to report the
2 ~~required~~ information REQUIRED BY THIS SECTION.

3 C. The board shall allow the reporter to transmit the required
4 information by electronic data transfer ~~if feasible or, if not feasible,~~
5 ~~on reporting forms as prescribed by the board.~~ The reporter shall submit
6 the required information ~~once each day~~ WITHIN ONE BUSINESS DAY AFTER THE
7 DATE THE PRESCRIPTION WAS SOLD. IF THERE IS NO INFORMATION TO REPORT, THE
8 REPORTER SHALL REPORT ZERO AS A TRANSACTION.

9 ~~D. A dispenser who does not have an automated recordkeeping system~~
10 ~~capable of producing an electronic report in the established format may~~
11 ~~request a waiver from electronic reporting by submitting a written request~~
12 ~~to the board. The board shall grant the request if the dispenser agrees~~
13 ~~in writing to report the data by submitting a completed universal claim~~
14 ~~form as prescribed by the board by rule.~~

15 ~~E. The board by rule may prescribe the prescription form to be used~~
16 ~~in prescribing a schedule II, III, IV or V controlled substance if the~~
17 ~~board determines that this would facilitate the reporting requirements of~~
18 ~~this section.~~

19 ~~F. D.~~ D. The reporting requirements of this section do not apply to
20 the following:

21 1. A controlled substance that is administered directly to a
22 patient.

23 2. A controlled substance that is dispensed by a medical
24 practitioner at a health care facility licensed by this state if the
25 quantity dispensed is limited to an amount adequate to treat the patient
26 for a maximum of seventy-two hours with not more than two seventy-two-hour
27 cycles within any fifteen-day period.

28 3. A controlled substance sample.

29 4. The wholesale distribution of a schedule II, III, IV or V
30 controlled substance. For the purposes of this paragraph, "wholesale
31 distribution" has the same meaning prescribed in section 32-1981.

32 5. A facility that is registered by the United States drug
33 enforcement administration as a narcotic treatment program and that is
34 subject to the recordkeeping provisions of 21 Code of Federal Regulations
35 section 1304.24.

36 Enroll and engross to conform

37 Amend title to conform

And, as so amended, it do pass

SELINA BLISS
CHAIRMAN

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